

# CAGRILINTIDE

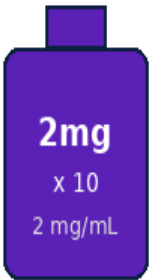
Long-Acting Amylin Analogue (Amylin Receptor Agonist)

Complete Injection & Dosing Guide -- All Vial Sizes

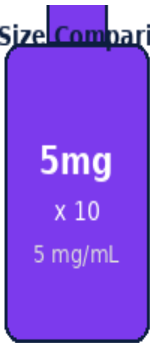
Purity: 99.8% | Endotoxin-Free | Heavy Metal-Free | Research Use Only

## Vial Size Comparison

Cagrilintide Vial Size Comparison -- SaiyanMed



**2mg**  
1mL BAC  
2 mg/mL



**5mg**  
1mL BAC  
5 mg/mL



**10mg**  
2mL BAC  
5 mg/mL

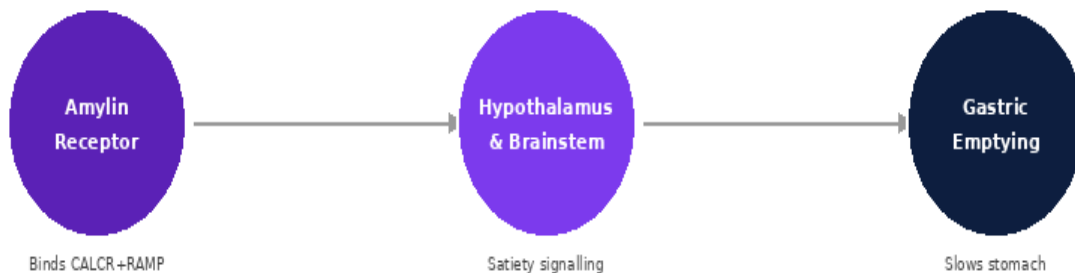
| Vial      | BAC Water | Concentration | Doses (0.3mg ea) | Examples per Kit (x10)                                         |
|-----------|-----------|---------------|------------------|----------------------------------------------------------------|
| 2mg x 10  | 1 mL      | 2.0 mg/mL     | 6                | 6 doses (0.3mg ea)   3 doses (0.6mg ea)   1 dose (2mg)         |
| 5mg x 10  | 1 mL      | 5.0 mg/mL     | 16               | 16 doses (0.3mg ea)   8 doses (0.6mg ea)   4 doses (1.2mg ea)  |
| 10mg x 10 | 2 mL      | 5.0 mg/mL     | 33               | 33 doses (0.3mg ea)   16 doses (0.6mg ea)   8 doses (1.2mg ea) |

## WHAT IS CAGRILINTIDE?

Cagrilintide is a **long-acting amylin analogue** designed to mimic the actions of amylin — a pancreatic hormone co-secreted with insulin that plays a critical role in post-meal satiety, gastric emptying regulation, and glucose homeostasis. Developed by Novo Nordisk, Cagrilintide acts on amylin receptors (CALCR + RAMP1/3) in the hypothalamus and brainstem, producing robust appetite suppression and body weight reduction.

In the landmark **CALM Phase 2 trial**, Cagrilintide 4.5mg alone achieved ~10.8% weight reduction over 26 weeks. In combination with semaglutide 2.4mg (the CagriSema combination, currently in Phase 3 REDEFINE trials), weight reduction exceeded 15% — with the REDEFINE-1 Phase 3 data showing up to 22.7% body weight reduction at 68 weeks.

### Cagrilintide Mechanism -- Amylin Receptor Pathway



| Receptor / Pathway                | Primary Action                                                                                            | Research Benefit                                                                                            |
|-----------------------------------|-----------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------|
| Amylin Receptor (CALCR + RAMP1/3) | Binds calcitonin receptor complexes in hypothalamus and area postrema; activates central satiety circuits | Potent appetite suppression independent of GLP-1 pathway; additive effect when combined with GLP-1 agonists |
| Gastric Motility                  | Slows gastric emptying; reduces post-meal glucose excursion; prolongs satiety after meals                 | Reduced caloric intake; blunted post-prandial glucose spikes; improved glycaemic control                    |
| Glucagon Suppression              | Post-prandial glucagon suppression; reduces hepatic glucose output                                        | Complementary glycaemic benefit; particularly relevant in Type 2 diabetes research models                   |
| Body Weight Centres               | Acts on hypothalamic POMC neurons and area postrema to reduce food intake and increase energy expenditure | Durable weight reduction; distinct from and complementary to GLP-1 receptor agonist mechanisms              |

## Key Research Benefits

| Benefit                                | Evidence & Research Context                                                                                                                                           |
|----------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Robust Weight Reduction</b>         | CALM Phase 2: 4.5mg monotherapy achieved ~10.8% weight loss over 26 weeks. REDEFINE Phase 3 (CagriSema): up to 22.7% at 68 weeks — comparable to surgical outcomes.   |
| <b>Complementary to GLP-1 Agonists</b> | Amylin and GLP-1 pathways are distinct and additive. CagriSema (Cagrilintide + Semaglutide) outperforms either agent alone across all weight reduction metrics.       |
| <b>Durable Appetite Suppression</b>    | Once-weekly dosing with ~7-day half-life maintains consistent receptor engagement without peak-trough fluctuations associated with shorter-acting agents.             |
| <b>Glycaemic Control</b>               | Post-prandial glucose suppression via gastric emptying delay and glucagon inhibition; relevant for Type 2 diabetes and metabolic syndrome research.                   |
| <b>Tolerability &amp; Lean Mass</b>    | GI side effects are manageable with slow escalation; emerging data suggests preferential fat mass reduction with lean mass preservation vs caloric restriction alone. |

## RECONSTITUTION PROTOCOL | DOSE CALCULATION

Always use **Bacteriostatic Water (BAC Water)** for reconstitution. Allow both the Cagrilintide vial and BAC Water to reach room temperature before mixing. Use a fresh insulin syringe for each reconstitution.

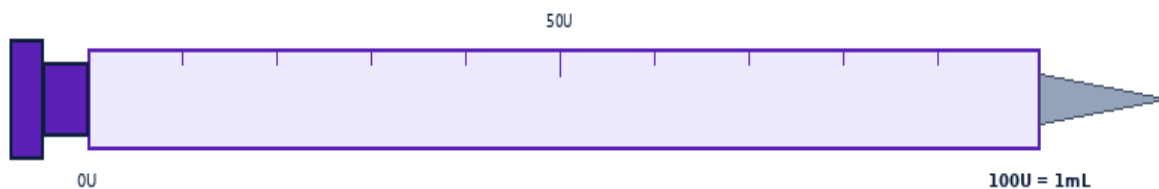


| Step | Action                   | Detail                                                                                                      |
|------|--------------------------|-------------------------------------------------------------------------------------------------------------|
| 1    | Gather supplies          | Cagrilintide vial, BAC Water vial, insulin syringes (29-31G), alcohol swabs, sharps container, date labels. |
| 2    | Clean vial stoppers      | Wipe rubber stoppers of BOTH vials with a fresh alcohol swab. Air-dry for 10-15 seconds before proceeding.  |
| 3    | Draw BAC Water           | Draw the correct volume per vial size (see Page 1 table). Expel all air bubbles before injection.           |
| 4    | Inject into peptide vial | Direct stream against the GLASS WALL — not onto the powder. Inject slowly and steadily over 10-15 seconds.  |
| 5    | Mix gently               | Roll vial gently between palms for 20-30 seconds. NEVER shake — agitation degrades the peptide.             |
| 6    | Inspect solution         | Must be clear and colourless. Discard immediately if cloudy, discoloured, or contains visible particles.    |
| 7    | Label and refrigerate    | Write reconstitution date on vial. Store at 2-8 deg C. Use within 28 days. Never freeze.                    |

### Reconstitution Quick Reference

| Vial Size | BAC Water | Resulting Concentration | Storage                | Use Within |
|-----------|-----------|-------------------------|------------------------|------------|
| 2mg       | 1 mL      | 2 mg/mL                 | 2-8 deg C refrigerated | 28 days    |
| 5mg       | 1 mL      | 5 mg/mL                 | 2-8 deg C refrigerated | 28 days    |
| 10mg      | 2 mL      | 5 mg/mL                 | 2-8 deg C refrigerated | 28 days    |

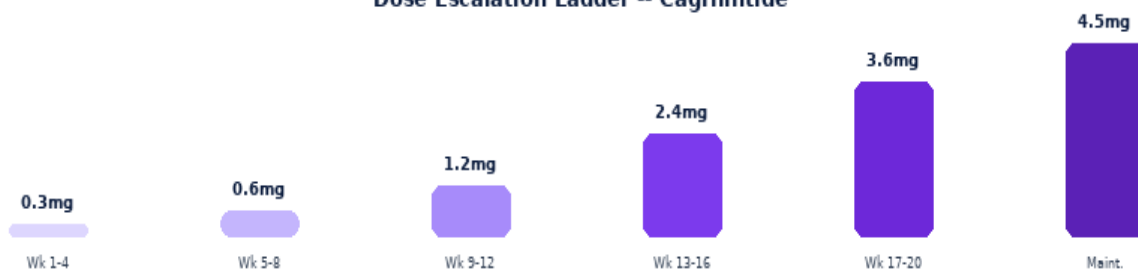
### DOSE CALCULATION GUIDE



### Insulin Syringe -- 1mL / 100 Units (29-31G needle)

**Formula:** Volume (mL) = Desired Dose (mg) / Concentration (mg/mL) | **Syringe Units:** Volume (mL) x 100 = Units on insulin syringe

### Dose Escalation Ladder -- Cagrilintide



**Vial: 2mg x 10** — Add 1 mL BAC Water → Concentration: 2.0 mg/mL

| Dose (mg)     | 0.3mg | 0.6mg | 1.2mg | 2.4mg  | 3.6mg  | 4.5mg  |
|---------------|-------|-------|-------|--------|--------|--------|
| Volume (mL)   | 0.150 | 0.300 | 0.600 | 1.200  | 1.800  | 2.250  |
| Syringe Units | 15.0U | 30.0U | 60.0U | 120.0U | 180.0U | 225.0U |

**Vial: 5mg x 10** — Add 1 mL BAC Water → Concentration: 5.0 mg/mL

| Dose (mg)     | 0.3mg | 0.6mg | 1.2mg | 2.4mg | 3.6mg | 4.5mg |
|---------------|-------|-------|-------|-------|-------|-------|
| Volume (mL)   | 0.060 | 0.120 | 0.240 | 0.480 | 0.720 | 0.900 |
| Syringe Units | 6.0U  | 12.0U | 24.0U | 48.0U | 72.0U | 90.0U |

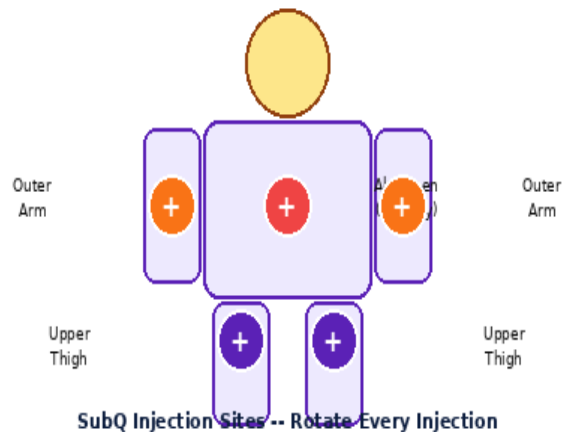
**Vial: 10mg x 10** — Add 2 mL BAC Water → Concentration: 5.0 mg/mL

| Dose (mg)     | 0.3mg | 0.6mg | 1.2mg | 2.4mg | 3.6mg | 4.5mg |
|---------------|-------|-------|-------|-------|-------|-------|
| Volume (mL)   | 0.060 | 0.120 | 0.240 | 0.480 | 0.720 | 0.900 |
| Syringe Units | 6.0U  | 12.0U | 24.0U | 48.0U | 72.0U | 90.0U |

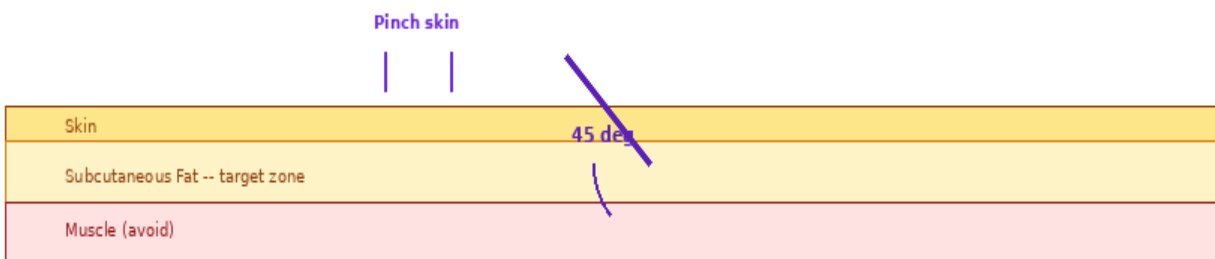
**Important:** Cagrilintide doses above 4.5 mg exceed Phase 3 trial doses. Always escalate in 0.3 mg increments every 4 weeks. Never exceed one injection per week.

## INJECTION TECHNIQUE

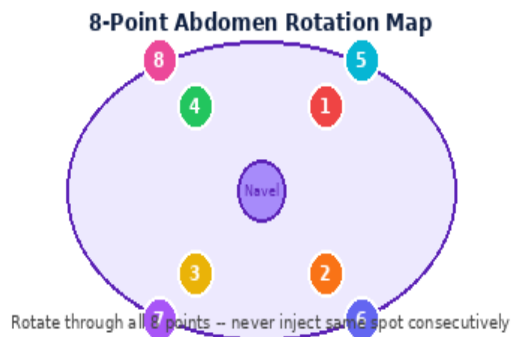
| Step | Action               | Detail                                                                                                              |
|------|----------------------|---------------------------------------------------------------------------------------------------------------------|
| 1    | Wash hands           | Wash with soap and warm water for 20 seconds. Dry completely.                                                       |
| 2    | Gather materials     | Insulin syringe (29-31G x 5/16"), Cagrilintide vial, alcohol swabs, sharps container.                               |
| 3    | Clean injection site | Wipe chosen site in a circular motion with an alcohol swab. Allow 10-15 seconds to dry completely before injecting. |
| 4    | Draw the dose        | Draw the correct volume per dose table. Tap syringe and expel all air bubbles before proceeding.                    |
| 5    | Pinch and insert     | Pinch 2-3 cm of skin. Insert needle at 45 degrees. Release pinch. Inject slowly over 5-10 seconds.                  |
| 6    | Withdraw and press   | Remove at same 45-degree angle. Press site gently 5-10 sec with dry swab. Do NOT rub — rubbing displaces the dose.  |
| 7    | Safe disposal        | Place needle immediately into sharps container. Never recap, reuse, or leave needles exposed.                       |



### 45-Degree SubQ Injection Angle



## SITE ROTATION & DOSING PROTOCOL



### Weekly Dosing Schedule -- Same Day Each Week



Choose any consistent day (Mon shown). 1 injection per week only.

| Phase       | Weeks      | Dose   | Frequency  | Notes                                                             |
|-------------|------------|--------|------------|-------------------------------------------------------------------|
| Starter     | 1-4        | 0.3 mg | 1x / week  | Assess GI tolerance. Nausea is common — inject with food.         |
| Step 2      | 5-8        | 0.6 mg | 1x / week  | Increase only if 0.3 mg is well tolerated for 4 weeks.            |
| Step 3      | 9-12       | 1.2 mg | 1x / week  | First clinically significant dose. Monitor appetite and weight.   |
| Step 4      | 13-16      | 2.4 mg | 1x / week  | Strong appetite suppression expected. Track caloric intake.       |
| Step 5      | 17-20      | 3.6 mg | 1x / week  | Escalate only if prior dose tolerated for full 4 weeks.           |
| Maintenance | 20+        | 4.5 mg | 1x / week  | Maximum Phase 3 trial dose. Titrate down if side effects persist. |
| Rest Period | Post-cycle | 0 mg   | 4-8 wk off | Allow receptor sensitivity to reset before repeating cycle.       |

**Administration tip:** Inject Cagrilintide with or shortly after a meal — nausea is more pronounced when injected in a fasted state, especially during the early escalation phase.

**CagriSema stacking note:** When combining with Semaglutide, inject each compound separately at different sites on the same day. Do not mix in the same syringe.

## SIDE EFFECTS, STORAGE & GOLDEN RULES

| Side Effect                   | Likelihood  | Management                                                                        |
|-------------------------------|-------------|-----------------------------------------------------------------------------------|
| Nausea                        | Very Common | Inject with food. Small, low-fat meals. Avoid strong smells on injection day.     |
| Reduced Appetite              | Very Common | Expected therapeutic effect. Maintain adequate protein and micronutrient intake.  |
| Vomiting                      | Common      | Usually dose-related. Hold escalation until resolved. Stay hydrated.              |
| Diarrhoea                     | Common      | Typically transient. Stay hydrated. Reduce dose if persistent.                    |
| Constipation                  | Occasional  | Increase fluid and dietary fibre intake. Usually self-resolving.                  |
| Injection site reaction       | Occasional  | Rotate sites systematically. Use fresh 29-31G needle each time.                   |
| Hypoglycaemia (if on insulin) | Rare        | Monitor blood glucose when combining with insulin. Reduce insulin dose as needed. |
| Tachycardia / Palpitations    | Rare        | Stop dosing. Seek medical advice before resuming.                                 |

**STOP & SEEK MEDICAL ADVICE: Severe persistent abdominal pain (rule out pancreatitis)**

**STOP & SEEK MEDICAL ADVICE: Neck lump, hoarseness, or difficulty swallowing (thyroid concern)**

**STOP & SEEK MEDICAL ADVICE: Severe allergic reaction: hives, facial swelling, breathing difficulty**

**STOP & SEEK MEDICAL ADVICE: Resting heart rate increase > 20 bpm above personal baseline**

**STOP & SEEK MEDICAL ADVICE: Persistent severe vomiting or inability to keep fluids down**

**STOP & SEEK MEDICAL ADVICE: Jaundice or dark urine (hepatic concern)**

**Absolute Contraindications: Personal or family history of medullary thyroid carcinoma or MEN2 syndrome, active or history of pancreatitis, pregnancy or lactation. Not for use in individuals under 18.**

## Storage Guidelines

| State                  | Temperature    | Duration   | Notes                                                    |
|------------------------|----------------|------------|----------------------------------------------------------|
| Unreconstituted powder | 2-8 deg C      | Per expiry | Original vial. Dry, away from light and heat.            |
| Reconstituted solution | 2-8 deg C      | 28 days    | Label with reconstitution date. Inspect before each use. |
| BAC Water (opened)     | 2-8 deg C      | 28 days    | Label with opening date.                                 |
| Short-term transport   | Below 25 deg C | Max 72 hr  | Insulated bag + ice packs. No direct ice contact.        |

## Golden Rules — Cagrilintide

1. Always start at 0.3 mg and escalate in 0.3 mg steps every 4 weeks — no skipping.
2. Inject with or after food — nausea is markedly worse when injected in a fasted state.
3. One injection per week only. Maximum dose is 4.5 mg per week in research protocols.
4. Never inject the same site twice in a row — rotate through all 8 marked abdominal points.
5. Use only BAC Water for reconstitution. Never use saline or plain sterile water.

6. Swirl gently to mix — NEVER shake. Agitation denatures the peptide.
7. Label every vial with the reconstitution date. Discard after 28 days.
8. Store at 2-8 deg C at all times. Never freeze reconstituted peptide.
9. Contraindicated in thyroid cancer history, MEN2, pancreatitis, or pregnancy.
10. If combining with Semaglutide (CagriSema), inject each compound separately at different sites.

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DISCLAIMER: This document is produced by SaiyanMed for informational and research reference purposes only. Cagrilintide is an investigational research compound. This guide does not constitute medical advice and must not be used to guide self-administration or human therapeutic use. Consult a qualified medical professional before beginning any peptide research protocol. SaiyanMed assumes no liability for misuse of this product or information contained herein.

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